

Amendment No. 1 to SB2155

Bailey
Signature of Sponsor

AMEND Senate Bill No. 2155*

House Bill No. 2619

by deleting all language after the enacting clause and substituting:

SECTION 1. Tennessee Code Annotated, Section 56-7-3706, is amended by deleting subsections (a) and (b) and substituting:

(a) A prior authorization required for an outpatient healthcare service to treat a chronic condition of an enrollee remains valid for at least twelve (12) months after the healthcare professional or provider receives the prior authorization approval, unless the clinical criteria in § 56-7-3707 provides otherwise.

(b) A prior authorization required for a prescription drug to treat a chronic condition of an enrollee remains valid for at least twelve (12) months after the healthcare professional or provider receives the prior authorization approval, unless the clinical criteria in § 56-7-3707 provides otherwise.

SECTION 2. Tennessee Code Annotated, Section 56-7-3703, is amended by adding the following as a new subsection:

(d)

(1) A healthcare professional with the same education and training as the healthcare professional requesting the prior authorization of a healthcare service shall make an adverse determination of medical necessity.

(2) Before making an adverse determination under subdivision (d)(1), the reviewing healthcare professional must review:

(A) The treating healthcare professional's recommendation;

(B) The enrollee's medical record or other clinical history; and

(C) The enrollee's individual clinical circumstances.

(3) A pharmacist may perform a medical necessity determination for any drug except a specialty drug. As used in this subdivision (d)(3), "specialty drug" means a high-cost, complex medication that is designed to treat a chronic, rare, or complex condition.

SECTION 3. Tennessee Code Annotated, Title 56, Chapter 7, Part 10, is amended by adding the following as new sections:

56-7-1022. Credit card payments.

(a) When initiating or changing the type of payment to a healthcare provider using electronic funds transfer payments, including virtual credit card payments, a health insurance entity or the health insurance entity's vendor must:

(1) Notify the healthcare provider of any fees associated with a particular payment method;

(2) Advise the healthcare provider of the available payment methods;
and

(3) Provide clear instructions to the healthcare provider on how to select an alternative payment method.

(b) A health insurance entity or the health insurance entity's vendor that initiates or changes the type of payment to a healthcare provider for healthcare electronic funds transfer and remittance advice transactions under 45 CFR 162.1601 and 45 CFR 162.1602 shall not charge a fee solely to transmit the payment to the healthcare provider unless the healthcare provider has consented to the fee.

(c) A healthcare provider's agent may charge reasonable fees for transaction management, data management, portal services, and other value-added services related to transactions under 45 CFR 162.1601 and 45 CFR 162.1602, in addition to any bank transmission fee.

56-7-1023. Medical complexity considerations in reimbursement.

A health insurance entity's contracts and reimbursement policies issued, delivered, entered into, amended, or renewed on or after April 30, 2027, must take into account a patient's medical complexity and comorbidity and must reimburse healthcare providers for covered services, procedures, and devices provided to the patient that are medically necessary.

SECTION 4. Tennessee Code Annotated, Title 56, Chapter 7, is amended by adding the following as a new part:

56-7-3901. Part definitions.

(a) As used in this part:

(1) "Commissioner" means the commissioner of commerce and insurance;

(2) "Downcoding":

(A) Means:

(i) The adjustment of a claim submitted to a health insurance entity to a less complex or lower cost procedure code; or

(ii) A reduction of billed service levels made by a health insurance entity, including, but not limited to, diagnosis-related group assignments, that reduces payment below the amount originally billed by the healthcare provider; and

(B) Does not mean a recoupment, as defined in § 56-7-110;

(3) "Health insurance entity" has the same meaning as defined in § 56-7-109; and

(4) "Healthcare provider" has the same meaning as defined in § 56-61-102 and includes a facility licensed under title 33.

56-7-3902. Prohibition on downcoding insurance claims.

A health insurance entity shall not downcode a claim that contains a current procedural terminology evaluation and management code submitted by a healthcare provider unless:

- (1) An employee or contractor of the health insurance entity makes the downcoding determination in accordance with the American Medical Association's current procedural terminology manual evaluation and management documentation criteria and the centers for medicare and medicaid services guidelines for reporting evaluation and management codes, after a documented clinical review of the medical records supporting the claim; and
- (2) The health insurance entity complies with the notification requirements in § 56-7-3904.

56-7-3903. Protections for patients with chronic conditions.

A health insurance entity shall not use downcoding practices in a targeted or discriminatory manner against a healthcare provider that routinely treats patients with complex or chronic conditions.

56-7-3904. Notification requirements for downcoded claims.

(a) A health insurance entity may downcode a claim submitted by a healthcare provider only after providing notice to the healthcare provider that the entity intends to downcode the claim, requesting from the healthcare provider the medical records that support the claim, and reviewing the medical records supporting the claim.

(b) If a health insurance entity intends to downcode a claim, then the health insurance entity must request from the submitting healthcare provider the medical records associated with such claim.

(c) A healthcare provider shall submit medical records requested by a health insurance entity pursuant to subsection (b) within fifteen (15) days after the date of the request.

(d) A health insurance entity that receives medical records pursuant to subsection (c) shall send a written acknowledgement of receipt of such records to the submitting healthcare provider.

(e) A health insurance entity shall pay a claim within twenty-one (21) days after the date the health insurance entity sends a written acknowledgement of receipt to a healthcare provider pursuant to subsection (d).

(f) If a healthcare provider does not submit to a health insurance entity the medical records requested pursuant to subsection (b), then the health insurance entity may downcode the claim and must provide notice as described in subsection (g).

(g) If a health insurance entity downcodes a claim after reviewing the medical records, then the health insurance entity must notify the healthcare provider in a remittance advice that includes:

- (1) The specific reasons for the downcoding, including reference to the clinical criteria used to justify the downcoding;
- (2) The original and revised service codes and payment amounts; and
- (3) Notice of the right to appeal under § 56-7-3905.

56-7-3905. Appeal process for downcoded claims.

(a) When a health insurance entity downcodes a claim submitted by a healthcare provider, the health insurance entity must provide the healthcare provider with:

- (1) Electronic notice to the email address listed in the healthcare provider's contract, or any subsequent contact update made to such contract, of the right to appeal the portion of the claim that was downcoded;
- (2) Instructions on how to file an appeal; and
- (3) The name, email address, and telephone number for the individual or department responsible for the appeal.

(b) The health insurance entity shall acknowledge receipt of an appeal in writing within five (5) business days after receipt.

(c) A health insurance entity may have multiple levels of appeals, not to exceed three (3) levels. The determination of at least one (1) level of appeal must be made by a healthcare professional with the same or a higher level of licensure as the treating healthcare professional. A health insurance entity shall only downcode a claim after reviewing the treating healthcare professional's recommendation, the enrollee's medical record or other clinical history, and the enrollee's individual clinical circumstances.

(d) The health insurance entity shall render a decision on an appeal no later than thirty (30) days after the date the healthcare provider files the appeal.

(e) If the health insurance entity determines that the claim should not have been downcoded, then the health insurance entity must pay the healthcare provider within fifteen (15) business days after the determination with any interest due to the healthcare provider pursuant to § 56-7-109.

(f) If the health insurance entity fails to render a decision within the thirty-day time period in subsection (d), then the claim is deemed approved and payable as originally submitted. The health insurance entity shall pay the healthcare provider within fifteen (15) days after the expiration of the thirty-day time period in subsection (d).

(g) A healthcare provider may appeal in batches of similar claims involving substantially similar downcoding issues by a health insurance entity.

(h) If a health insurance entity has a level of appeal that includes arbitration, then the health insurance entity must not require a healthcare provider to have more than one (1) arbitrator. Such appeal level must include an option for the healthcare provider to participate virtually in arbitration.

56-7-3906. Enforcement of part.

(a) The commissioner may assess a civil penalty for a violation of this part in accordance with § 56-2-305.

(b) This part must be construed to promote prompt and full payment for healthcare services rendered. A health insurance entity shall not evade, delay, or otherwise avoid liability under this part through internal procedures, policy language, or administrative practices.

(c) The commissioner may, upon finding that a contract provision, policy term, or administrative practice waives, limits, or circumvents the protections of this part, order such provision to be unenforceable as against public policy.

SECTION 5. If any provision of this act or its application to any person or circumstance is held invalid, then the invalidity does not affect other provisions or applications of the act that can be given effect without the invalid provision or application, and to that end, the provisions of this act are severable.

SECTION 6. The headings in this act are for reference purposes only and do not constitute a part of the law enacted by this act. However, the Tennessee Code Commission is requested to include the headings in any compilation or publication containing this act.

SECTION 7.

(a) For the purpose of promulgating rules, this act takes effect upon becoming a law, the public welfare requiring it.

(b) For all other purposes, this act takes effect April 30, 2027, the public welfare requiring it.